OCD-02 | For therapists | OCD, ages 8 and over

Assessment and understanding the OCD cycle together

1 | Purpose, fit and boundaries

Purpose of this unit

Gather a brief account of a situation, unwanted experience, response and functional effects. This draft supports conversation and clinical formulation, without scoring or diagnosis.

Choice and adaptation

Choose this sheet following professional assessment and with consent. The person may skip questions, answer aloud, or choose another tool. This is proposed original material for review; it provides no score, diagnosis, or promise of an outcome.

Boundaries specific to this unit

Do not infer intent or danger from an unwanted thought. The therapist separately assesses context, distress and actual risk. Do not demand details, continuous thought counts or repeated checks for certainty.

Reusing existing material

OCD-01 prepares for a session; this unit adds the possible role of overt or covert responses in the cycle. ASM-A01 and CORE-02 can support description but do not diagnose OCD.

A fictional example for discussion

Fictional example: Uri repeatedly delays submitting work. He briefly describes doubt and repeated checking. The therapist clarifies actual task requirements and what the response seeks to achieve, without diagnosing from the example.

> Draft for administrator review. Clinical review and final rights approval are still pending.

Mindful Path

OCD-02 | For therapists | OCD, ages 8 and over

Working together with the worksheet

2 | A conversation about one situation

Opening

Choose a situation the person wants to explore. It need not be the hardest situation. Check that the question is understood and the wording fits the person; their account comes before the therapist's interpretation.

1 | Question for discussion

What specific situation fits shared formulation?

2 | Question for discussion

What unwanted experience occurred and how did I interpret it?

3 | Question for discussion

What was I trying to achieve through an overt or covert response?

4 | Question for discussion

What immediate benefit and later cost did the response have?

5 | Question for discussion

What circumstances or needs should be clarified before concluding?

Closing

Summarize what became clearer and what remains open. Use the existing SES-S01 and SES-S02 for the session framework. There is no need to repeat an assessment or plan already recorded.

Possible difficulties

If completion becomes repeated correction, use a spoken description. If distinguishing thought from response is hard, help with wording without requiring a perfect answer.

Mindful Path

OCD-02 | For therapists | OCD, ages 8 and over

Adapting for four age groups

3 | Age is a starting point, not an assumption about ability

CH | Children aged 8–11 only

Ages 8–11 only. Read one question and allow a brief or spoken response. Parents add separate observations without deciding the child's thoughts; do not test reporting accuracy. Distinguish the child's account from adult observation.

Offer one question at a time with little writing. Adult support must not replace the child's own voice.

TE | Adolescents, approximately ages 12 to 17

Ages 12–17. Agree what stays in the session and what is shared with family or school, subject to safeguarding duties. Embarrassing thought content need not be disclosed. Describe the thought's function without writing its content if preferred.

Agree in advance what will be shared and with whom, and explain the privacy limits in this setting. Do not promise absolute confidentiality.

AD | Adults

Connect work to chosen goals, family, employment and existing care. Distinguish rituals, practical needs and value-consistent behavior. Separate assessment determines whether the response relates to OCD.

OA | Older adults

Preserve adult language, choice and privacy. Assess access, health and desired support without automatically attributing new changes to OCD or assuming reduced ability. Sudden change or safety needs are not classified as rituals from this form.

Offer reading support, time, and conversation according to preference. Choose help from another person together with the participant, respecting their privacy.

> Each age version needs its own review. Approval of one version does not automatically approve the others.

Mindful Path

OCD-02 | For therapists | OCD, ages 8 and over

Practice, review and next steps

4 | Shared choice and learning from the attempt

Before offering practice sheet P

The P sheet proposes recording decisions and learning within treatment, not ERP instructions. The therapist determines suitability, action, scope and support. Without an agreed plan, use only for discussion. Do not begin exposure or change response prevention based on this sheet.

At the next session

Review new understanding of functioning and separate assessment needs. Ask whether tracking helped conversation or became another check; adjust the recording method, not the desired answer.

Professional and technical records

Each version has a separate review record covering age suitability, originality, rights, and fictional filling. Reviewer and date fields remain blank until assigned. File checks or a code merge are not clinical approval. Clinical information stays only in the authorized setting, outside the code repository.

Background references

The following links were used for background only. Questions and adaptations were written afresh for the expansion plan; no questionnaire, scale, or protocol was copied. Adult sources do not validate the child versions. This material makes no claim about session count or treatment effectiveness.

Treatment-manual mapping status

Mapping against a full OCD manual and professional approval remain pending. This is original support material for review, not an approved ERP manual or its replacement.

www.nice.org.uk/guidance/cg31/chapter/Recommendations
iocdf.org/about-ocd/ocd-treatment-guide/erp/
kids.iocdf.org/professionals/md/treatment/

Mindful Path